

Q Will my health insurance cover a midwife instead of a doctor?

It depends on your health insurance company and the state where you live—there are no national guidelines. If you're eager to see a midwife, call your insurer and check before you go to your first appointment. Only a small number cover costs for appointments with midwives and births at birthing centers, rather than hospitals. If you want to pay out of pocket to see a midwife, it should be considerably less than if you paid out of pocket for a doctor, partly because midwives tend to order fewer tests than doctors.

Q What blood tests are taken at my prenatal appointments?

A range of tests can check for blood type, infections, genetic diseases, and anemia. Your doctor may take samples for the following:

» **Your blood type and rhesus status** Tested once in early pregnancy. Your blood type (A, B, AB, or O) will be recorded in your records in case for any reason you need a transfusion during delivery. If you are rhesus negative (see p.82) you will need one or two additional injections.

» **Hepatitis B** Tested once, in early pregnancy. This viral infection can cause liver damage in the baby, and the baby needs treatment at birth.

» **Anemia** Tested at the first appointment and at 28 weeks. A full blood count can suggest anemia if your hemoglobin levels are low. It also tests levels of blood folate and vitamin B₁₂, blood platelets, and white cell count (to check that you aren't fighting an infection).

» **Rubella immunity** Tested once in early pregnancy. The rubella virus can cause serious congenital abnormalities in babies (see p.139). If you're not immune, you can't be vaccinated in pregnancy so your doctor will advise on how to minimize your risk of exposure in pregnancy.

» **HIV and syphilis** Tested once in early pregnancy. These diseases pose a significant risk to your baby if you don't know about them—but your baby can be protected if you do. You have the right to refuse an HIV/AIDS test.

» **Sickle-cell disease and thalassemia** Tested once in early pregnancy. This is a test for genetic blood disorders that can be passed to the baby. Usually offered to those at risk of being carriers.

» **Other tests** If you're at high risk, you may have blood tests for hepatitis C, vitamin D deficiency, toxoplasmosis, and for chicken pox immunity if you're not sure of your history.

Q Will I be screened for Group B streptococcus (or GBS)?

Yes. Women are tested for GBS late in pregnancy, typically between 35 and 37 weeks. You'll have two swabs, one vaginal and one rectal, and if you carry the infection, you may be offered intravenous antibiotics during your labor, or the baby will be given antibiotics as soon as she is born. If you were GBS-positive during a previous pregnancy, your doctors will treat you with antibiotics again this time.

Q Why do I need to provide a urine sample at each appointment?

Your doctor tests urine for protein (high levels indicate high blood pressure, preeclampsia, urinary tract infection, or kidney disease). If you have gestational diabetes, your doctor may also test your urine to ensure that your condition is under control. Plan to give a sample at each visit.

Q Will I need a whooping cough vaccine? Why?

You will be offered a whooping cough vaccine between 27 and 36 weeks. Cases of whooping cough have gone up in recent years. Babies are routinely vaccinated against the virus at two months, but before this time they're vulnerable to infection. If you are vaccinated in pregnancy the immunity crosses the placenta and protects your baby in the weeks after birth.

Q Why is my blood pressure measured at every checkup? What are they looking for?

Pregnancy-induced hypertension (high blood pressure) (see p.144) can restrict the flow of blood to your baby, interfering with his growth. Your doctor will take your blood pressure throughout your pregnancy in order to be vigilant.

If your blood pressure rises too much, it also carries risks for you (the same risks as high blood pressure in nonpregnant people). It can also be a symptom of preeclampsia (see p.144), along with protein in your urine. Average pregnancy blood pressure is actually a little lower than normal nonpregnancy blood pressure for most of pregnancy, because hormones dilate your arteries to deal with the extra blood volume your pregnant body creates to grow your baby. By the end of pregnancy, you have about an extra 1¾ pints (1 liter) of blood pumping around your body.

Blood pressure readings These have an upper number (systolic) that denotes the pressure in your arteries when your heart pumps, and a lower number (diastolic) for the pressure between pumps when your heart rests.

